PREOPERATIVE DIAGNOSIS: Symptomatic mesioangular bony impacted mandibular right third molar (tooth #32) with recurrent pericoronitis.

POSTOPERATIVE DIAGNOSIS: Bony impacted tooth #32 cleanly removed.

PROCEDURE PERFORMED: Surgical extraction of bony impacted mandibular right third molar (tooth #32) with bone removal and tooth sectioning.

OPERATIVE SUMMARY:

The patient was placed in the dental chair. Local anesthesia was achieved utilizing right inferior alveolar nerve block and long buccal nerve infiltration with 3.6 mL of 2% lidocaine with 1:100,000 epinephrine. Profound pulpal and soft tissue anesthesia was confirmed.

A standard triangular mucoperiosteal envelope flap was reflected using a periosteal elevator from the distobuccal line angle of tooth #31 extending along the external oblique ridge. Under continuous sterile saline irrigation, buccal and distal alveolar bone guttering was executed using a surgical round bur. The tooth was sectioned horizontally at the cementoenamel junction using a 702 fissure bur. The crown fragment was mobilized and removed with a straight elevator, followed by clean delivery of both mesial and distal roots. The extraction socket was aggressively debrided of follicle, curetted, and irrigated with sterile saline. The mucoperiosteal flap was reapproximated and secured with two 3-0 silk interrupted sutures. Hemostasis achieved with biting pressure on sterile gauze.